

Clinical Study Summary Report (CSSR)

Document Status: Final | Version: 1.0 | Date: 01-APR-2026

1. Administrative & Study Identification

Full Study Title:	A Phase 2 Multi-Cohort, Open-Label, Multi-Center Clinical Study Evaluating the Efficacy and Safety of Disitamab Vedotin (RC48-ADC) Alone or in Combination With Pembrolizumab in Subjects With Locally-Advanced Unresectable or Metastatic Urothelial Carcinoma That Expresses HER2
Short Title:	A Study of Disitamab Vedotin Alone or With Pembrolizumab in Urothelial Cancer That Expresses HER2
Protocol Number:	48603868157989883
NCT Number:	NCT04879329
Sponsor Name:	Pfizer
Investigational Product:	pembrolizumab
Phase:	PHASE2
Medical Monitor:	[Not Provided in Posting]

2. Study Synopsis & Rationale

2.1 Study Objective

Primary Objective:	1. Confirmed Objective Response Rate (cORR) per Response Evaluation Criteria in Solid Tumors (RECIST) version 1.1 (v1.1) by blinded independent central review (BICR) (Cohorts A, B, C, and G): The proportion of participants with confirmed complete response (CR) or partial response (PR) according to RECIST v1.1 2. Incidence of adverse events (AEs) (Cohorts D and E): Any untoward medical occurrence in a clinical investigational participant administered a medicinal product and which does not necessarily have a causal relationship with this treatment. 3. Incidence of dose alterations (Cohorts D and E)
Secondary Obj.:	1. cORR per RECIST v1.1 by investigator assessment (Cohorts A, B, C, and G) 2. Confirmed Duration of Response (DOR) per RECIST v1.1 by BICR (Cohorts A, B, C, and G) 3. Confirmed DOR per RECIST v1.1 by investigator assessment (Cohorts A, B, C, and G)

2.2 Background & Rationale

This study is being done to see if a drug called disitamab vedotin, alone or with pembrolizumab, works to treat HER2 expressing urothelial cancer. It will also test how safe the drug is for participants.

3. Study Design & Methodology

Design Framework:	Type: INTERVENTIONAL, Phase: PHASE2, Status: RECRUITING
Target N:	372

4. Subject Selection (Eligibility Criteria)

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4.1 Inclusion Criteria

Cohorts A and B

- * Histopathologically-confirmed, locally-advanced, unresectable or metastatic urothelial cancer (LA/mUC), including UC originating from the renal pelvis, ureters, bladder, or urethra
- * Participants must have received only 1 or 2 lines of prior systemic treatment for LA/mUC, including 1 line of platinum-containing chemotherapy
- * At least one measurable lesion by investigator assessment based on RECIST version 1.1.
- * HER2-expression status determined by the central laboratory to be IHC 1+, 2+ or 3+, in the provided tumor sample
- * Eastern Cooperative Oncology Group (ECOG) performance status score of 0 or 1

Cohort C

- * Histopathologically-confirmed LA/mUC, including UC originating from the renal pelvis, ureters, bladder, or urethra
- * No prior systemic therapy for LA/mUC
 - * Neoadjuvant or adjuvant therapy, including PD-(L)1 inhibitors, is acceptable, if disease recurrence/progression occurred more than 12 months after the last dose of systemic therapy
- * At least one measurable lesion by investigator assessment based on RECIST v1.1.
- * Participant is eligible to receive cisplatin- or carboplatin- containing chemotherapy per investigator evaluation
- * HER2-expression status determined by the central laboratory to be IHC 1+, 2+ or 3+, on the provided tumor tissue sample
- * ECOG performance status of 0, 1, or 2

Cohort D

- * Histopathologically-confirmed LA/mUC, including UC originating from the renal pelvis, ureters, bladder, or urethra
- * Based on a participant's eligibility to receive treatment with standard of care therapies in Japan, participants must have received all of the following lines of therapy for LA/mUC:
 - * a. One prior line of platinum-containing chemotherapy.
 - * b. Prior therapy with PD-(L)1 inhibitors as (neo)adjuvant therapy, first-line maintenance therapy or as second line treatment.
 - * c. Prior enfortumab vedotin therapy.
- * At least one measurable lesion by investigator assessment based on RECIST v1.1.
- * ECOG performance status of 0 or 1

Cohort E

- * Histopathologically-confirmed LA/mUC, including UC originating from the renal pelvis, ureters, bladder, or urethra

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- * No prior systemic therapy for LA/mUC

- * Neoadjuvant or adjuvant therapy, including PD-(L)1 inhibitors, is acceptable, if disease recurrence/progression occurred more than 12 months after the last dose of systemic therapy.

- * At least one measurable lesion by investigator assessment based on RECIST v1.1.

- * Participant is eligible to receive cisplatin- or carboplatin- containing chemotherapy per investigator evaluation

- * HER2-expression status determined by the central laboratory to be IHC 1+, 2+ or 3+, in the provided tumor sample

- * ECOG performance status of 0 or 1

Cohort G

- * Histopathologically-confirmed, locally-advanced, unresectable or metastatic urothelial cancer (LA/mUC), including UC originating from the renal pelvis, ureters, bladder, or urethra

- * Participants must have received only 1 or 2 lines of prior systemic treatment for LA/mUC, including 1 line of therapy containing enfortumab vedotin as monotherapy or in combination with pembrolizumab

- * The last administration of enfortumab vedotin must be 90 days from the start of study treatment. Intervening therapies are allowed between the final dose of enfortumab vedotin and the start of disitamab vedotin.

- * At least one measurable lesion by investigator assessment based on RECIST version 1.1.

- * HER2-expression status determined by the central laboratory to be IHC 1+, 2+ or 3+, in the provided tumor sample

- * Eastern Cooperative Oncology Group (ECOG) performance status score of 0 or 1

4.2 Exclusion Criteria

Cohorts A and B

- * Known hypersensitivity to disitamab vedotin or any of their components

- * Prior antitumor treatment (including chemotherapy, radiotherapy, targeted therapy, immunotherapy etc.) within 2 weeks of start of study (defined as Cycle 1 Day 1 for Cohorts A and B)

- * Toxicity from a previous treatment has not returned to Grades 0 or 1 (except for Grade 2 alopecia)

- * Prior MMAE-based ADCs (eg, enfortumab vedotin) or HER2-directed therapy

- * Major surgery that has not fully recovered within 4 weeks prior to dose administration

- * Peripheral sensory or motor neuropathy ? Grade 2 at baseline

Cohort C

- * Known hypersensitivity to disitamab vedotin, pembrolizumab, or any of their components

- * Prior antitumor treatment (including chemotherapy, radiotherapy, targeted therapy, immunotherapy etc.) within 2 weeks of start of study defined as Cycle 1 Day 1 for the single-arm part of Cohort C and as randomization date for the randomized part of Cohort C)

- * Toxicity from a previous treatment has not returned to Grades 0 or 1 (except for Grade 2 alopecia)

- * Prior MMAE-based ADCs (eg, enfortumab vedotin) or HER2-directed therapy

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- * Major surgery that has not fully recovered within 4 weeks prior to dose administration
- * Peripheral sensory or motor neuropathy ? Grade 2 at baseline
- * Has a diagnosis of immunodeficiency or is receiving chronic systemic steroid therapy (in dosing exceeding 10 mg daily of prednisone equivalent) or any other form of immunosuppressive therapy within 7 days prior the first dose of study drug
- * Participants who have previously received any prior treatment with an agent directed to another stimulatory or co-inhibitory T cell receptor (including but not limited to CD137 agonists, CAR-T cell therapy, CTLA-4 inhibitors, or OX-40 agonists) are excluded.

Cohort D

- * Known hypersensitivity to disitamab vedotin or any of their components
- * Prior antitumor treatment (including chemotherapy, radiotherapy, targeted therapy, immunotherapy etc.) within 2 weeks of start of study (defined as Cycle 1 Day 1 for Cohort D)
- * Toxicity from a previous treatment has not returned to Grades 0 or 1 (except for Grade 2 alopecia)
- * Prior HER2-directed therapy
- * Any prior history of ? Grade 3 non-hematological AEs related to prior therapy
- * Major surgery that has not fully recovered within 4 weeks prior to dose administration
- * Peripheral sensory or motor neuropathy ? Grade 1 at baseline

Cohort E

- * Known hypersensitivity to disitamab vedotin, pembrolizumab, or any of their components
- * Prior antitumor treatment (including chemotherapy, radiotherapy, targeted therapy, immunotherapy etc.) within 2 weeks of start of study (defined as Cycle 1 Day 1 for Cohort E)
- * Toxicity from a previous treatment has not returned to Grades 0 or 1 (except for Grade 2 alopecia)
- * Any prior history of ? Grade 3 non-hematological AEs related to prior therapy
- * Prior MMAE-based ADCs (eg, enfortumab vedotin) or HER2-directed therapy
- * Major surgery that has not fully recovered within 4 weeks prior to dose administration
- * Peripheral sensory or motor neuropathy ? Grade 1 at baseline
- * Has a diagnosis of immunodeficiency or is receiving chronic systemic steroid therapy (in dosing exceeding 10 mg daily of prednisone equivalent) or any other form of immunosuppressive therapy within 7 days prior the first dose of study drug

Cohort G

- * Known hypersensitivity to disitamab vedotin or any of their components
- * Prior antitumor treatment (including chemotherapy, radiotherapy, targeted therapy, immunotherapy etc.) within 2 weeks of start of study (defined as Cycle 1 Day 1 for Cohort G)
- * Toxicity from a previous treatment has not returned to Grades 0 or 1 (except for Grade 2 alopecia)

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- * Prior HER2-directed therapy
- * Major surgery that has not fully recovered within 4 weeks prior to dose administration
- * Peripheral sensory or motor neuropathy ? Grade 2 at baseline

There are additional inclusion and exclusion criteria. The study center will determine if criteria for participation are met.

11. Study Identifiers & Governance

Primary ID: 48603868157989883

Registry Number: NCT04879329

Sponsor: Pfizer

Study Title: A Study of Disitamab Vedotin Alone or With Pembrolizumab in Urothelial Cancer That Expresses HER2

15. Sign-off & Approval

Principal Investigator: _____ Date: _____

Clinical Operations Lead: _____ Date: _____

Lead Statistician: _____ Date: _____